

BODY DECODE • FOUR EXPLANATIONS

Before You Accept It's Your Hormones

Four explanations for tired, wired and stuck. Only one of them ever gets checked.

Tired all the time. Awake at 3am. Body will not change.

You have almost certainly been given one explanation for that.

It is probably half right. Sixty per cent of the women who complete our profile are perimenopausal or past it, so hormones really are in the mix, and anyone telling you otherwise is selling something.

But here is what we watch happen. **The moment hormones get the blame, everything else stops being checked.** Nobody asks what your week actually costs you. Nobody asks whether you are eating enough for what you are doing. Nobody asks how long this has been going on, or whether it started before anything else changed.

You got an explanation, so the looking stopped.

This is not an argument that it is not hormonal. It is an argument that hormonal was the only thing anybody checked. There are four common explanations for the same three symptoms, most women have more than one running at once, and the plan is different for each.

1 The four, and how to read your ticks

1. **Oestrogen is falling.** The one you have already been given.
2. **The load is higher than the recovery.** Mistaken for the first one most often.
3. **You are eating for half the life you are living.** The one nobody self-identifies with.
4. **Your breathing is interrupting your sleep.** The one nobody screens for in women.

Each has five statements. **Three or more ticks in a section means it is worth taking seriously.** Not that you have it, and not instead of the others. It means that one has earned a proper look rather than being assumed or ignored.

Section four is different, and the threshold is lower. Two ticks there is enough to raise it with your GP, because the cost of missing it is higher than the cost of asking.

2 One · Oestrogen is falling

The explanation you have already been given, and often a real part of it.

- ☐ **It used to sit on your hips and thighs, and over the last few years it has moved to your middle.**
- ☐ **Your cycle changed before anything else did.** Shorter, heavier, less predictable.
- ☐ **Sleep went first in the second half of the month,** before it went every night.

- ☐ **Joints ache in the morning** in a way they never used to.
- ☐ **It came on over years, not months.**

Three or more ticked? Worth taking seriously.

What is actually going on. Oestrogen does more than reproduction. It influences where the body stores fat, how well it handles carbohydrate, how it regulates temperature, and how easily it holds sleep. As it falls, storage shifts away from the hips and thighs and toward the middle, which is why the location moves rather than simply increasing. Sleep usually goes first in the second half of the cycle, before it goes every night, because that is when the drop is steepest.

What changes if it is this one. The correction is not more effort, it is different timing. Training and food get organised around where you are in the month rather than against it. Strength work matters more than it used to, not less.

Three things worth doing this week. None of them are a plan, and none of them need anybody's permission.

1. **Put strength work in, and keep it heavy enough to be hard.** Falling oestrogen costs muscle and bone faster than anything else on this list, and strength is the one input that argues back.
2. **Protein at every meal, not just the one after training.** The amount your body can use at a sitting goes down with age, so spreading it matters more than it used to.
3. **Track when in the month the bad weeks fall.** Two cycles of noting it tells you more than any single blood test, and it is the thing a GP conversation actually needs.

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Two · The load is higher than the recovery

The one that gets mistaken for hormonal most often, because it produces the same 3am.

- ☐ **Your middle is where you have always carried it**, even at thirty.
- ☐ **You are wired at night and flat in the morning.**
- ☐ **Your arms and legs stayed lean while the middle filled out.**
- ☐ **You fall asleep fine, then wake around three and start thinking.**
- ☐ **It got worse in a period you could name.** A job, a loss, a move, a year.

Three or more ticked? Worth taking seriously.

What is actually going on, and why 3am specifically. Cortisol runs on a daily rhythm. It sits lowest around midnight, then climbs through the early hours so you are ready to wake. That rise happens in everyone, every night, including people who sleep straight through.

So the rise is not what wakes you. **What differs is the level it is rising from.** A body carrying sustained load tends to sit higher through the evening and overnight, so the same ordinary climb crosses the line

into being awake. The second half of the night is also naturally lighter, which is why it lands at three and four rather than eleven.

Be careful with the popular version of this. “A 3am cortisol spike” is not quite what happens. It is a gradual rise, it is normal, and it is not the problem on its own. And waking at 3am is a symptom all four explanations in this guide can produce, which is why *how* you wake matters more than *when*. Mind already running is this one. Waking hot is usually the first one. Waking gasping is the fourth.

What changes if it is this one. Nothing about the training gets harder. The work is in what surrounds it, because a body in protection will not spend what it does not think it can spare.

Three things worth doing this week.

1. **Fix the wake time, not the bedtime.** The rhythm anchors to when light hits your eyes, not when you turn the lamp off. Same time every day including the weekend, then daylight within about half an hour of getting up.
2. **Drop the intensity, keep the frequency.** Hard sessions add to the same load that is keeping you up. Same number of sessions, easier ones, for a fortnight.
3. **Put something between the day and bed that is not a screen.** Ten minutes. The point is a signal that the day has ended, and a phone is the opposite of one.

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Three · You are eating for half the life you are living

The one almost nobody self-identifies with, and the one that looks most like discipline.

- ☐ **You are the most careful person about food that you know.**
- ☐ **Your weight is stable and nothing changes anyway.**
- ☐ **You feel the cold more than you used to.**
- ☐ **Sessions that used to feel easy feel heavy**, and recovery takes days.
- ☐ **You eat least on the days you train hardest.**

Three or more ticked? Worth taking seriously.

What is actually going on. The body does not count calories, it reads availability. When what comes in stays below what the day costs for long enough, it protects rather than adapts: it turns down the things that are expensive and not immediately essential. Thyroid output eases off, reproductive hormones ease off, body temperature drops slightly, and the amount of energy burned at rest falls to match. That is why the cold hands and the flat sessions arrive together, and why the scales stop moving on less food rather than more.

What changes if it is this one. More, not less. Which is the hardest sentence in this guide to accept, because everything you have been told rewards the opposite.

Three things worth doing this week. No numbers here on purpose, and none of them are about eating less.

1. **Eat before the session, not only after it.** Training on nothing is the single most common version of this.
2. **Add an eating occasion rather than more food in one sitting.** A body that has been short for a while handles it better spread out.
3. **Eat most on the days you train hardest.** Most women do the opposite without noticing, because the hard days are also the busy ones.

A word of care. If food feels like something you are managing rather than eating, this guide is not the right tool and neither is another plan. Please talk to your GP.

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Four · Your breathing is interrupting your sleep

The one nobody screens for in women, because it does not look the way people expect.

Up to three quarters of women who have sleep apnoea have never been diagnosed. The screening was built around how it presents in men, so the picture people look for is snoring and daytime sleepiness. In women it more often shows up as fatigue that never lifts, waking in the early hours, low mood and morning headaches.

Which is the same list that gets called hormonal.

- ☐ **You wake with a headache more than once a week.**
- ☐ **You have been told you snore, or you wake with a dry mouth.**
- ☐ **You have woken gasping, or with your heart going.**
- ☐ **The tiredness does not lift after a genuinely good night.**
- ☐ **It has been going on for years, and it predates any hormonal change.**

What is actually going on. During sleep the airway narrows. Oxygen dips, and the brain briefly surfaces to reopen it and take a breath. That can happen dozens of times an hour without you remembering any of it. You are not getting to the deep stages at all, which is why a full eight hours changes nothing. The morning headache comes from the carbon dioxide that builds while breathing is interrupted.

This is the one we do not touch. We screen and refer, we do not diagnose. If you ticked two or more above, that is a conversation with your GP about a sleep study, and it is worth having before you spend another year on the other three. Nothing in this guide replaces that.

And no quick wins for this one, deliberately. The other three sections each end with three things worth trying. This one does not, because there is nothing you can do at home that fixes a narrowing airway, and a list here would only delay the conversation that actually helps. That absence is the advice.

If you ticked boxes in more than one

Most women do. That is not the guide failing, it is the actual situation.

Two of these commonly run together, and the order matters. A body that is under-fuelled and under-slept will not respond to a plan built for falling oestrogen, however correct that plan is on paper. **The one that is furthest from normal gets addressed first**, not the one you have heard the most about.

So if you ticked three in one section and five in another, the five goes first. And if section four is one of them, that conversation happens before any of it.

Which is the whole reason we assess before we prescribe. Across the women we have profiled, only **18 in 100** came out able to absorb a hard training plan. The other eighty-two get handed one anyway.

Find out which one is yours.

The free 14-Day Body Decode Challenge scores five foundations on day one and works out which of the four is driving it, so the plan is built for the body you are actually in.



bodyrecode.au/challenge